

The Global Burden of Mental Illness: Prevalence, Impact, and the Treatment Gap

Global data (World, 2023) • Our World in Data / IHME GBD

Why it matters: mental health shapes well-being, ability to work, and relationships — yet treatment is often lacking or poor quality.

 **1 in**
3
women
lifetime risk of major depression

 **1 in**
5
men
lifetime risk of major depression

Mental health conditions range widely in frequency and severity:

Common: depression, anxiety

Less common, severe: schizophrenia, bipolar

For policy, high-prevalence conditions drive large population impact, while severe disorders require specialized support to reduce disability and improve functioning.

Disease burden (DALYs per 100,000)

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Burden of disease from each category of mental illness (World, 2023)

Depressive disorders

694.6

Anxiety disorders

686.5

Schizophrenia

210.2

Bipolar disorder

94.6

Eating disorders

47.5

0

20
0

40
0

60
0

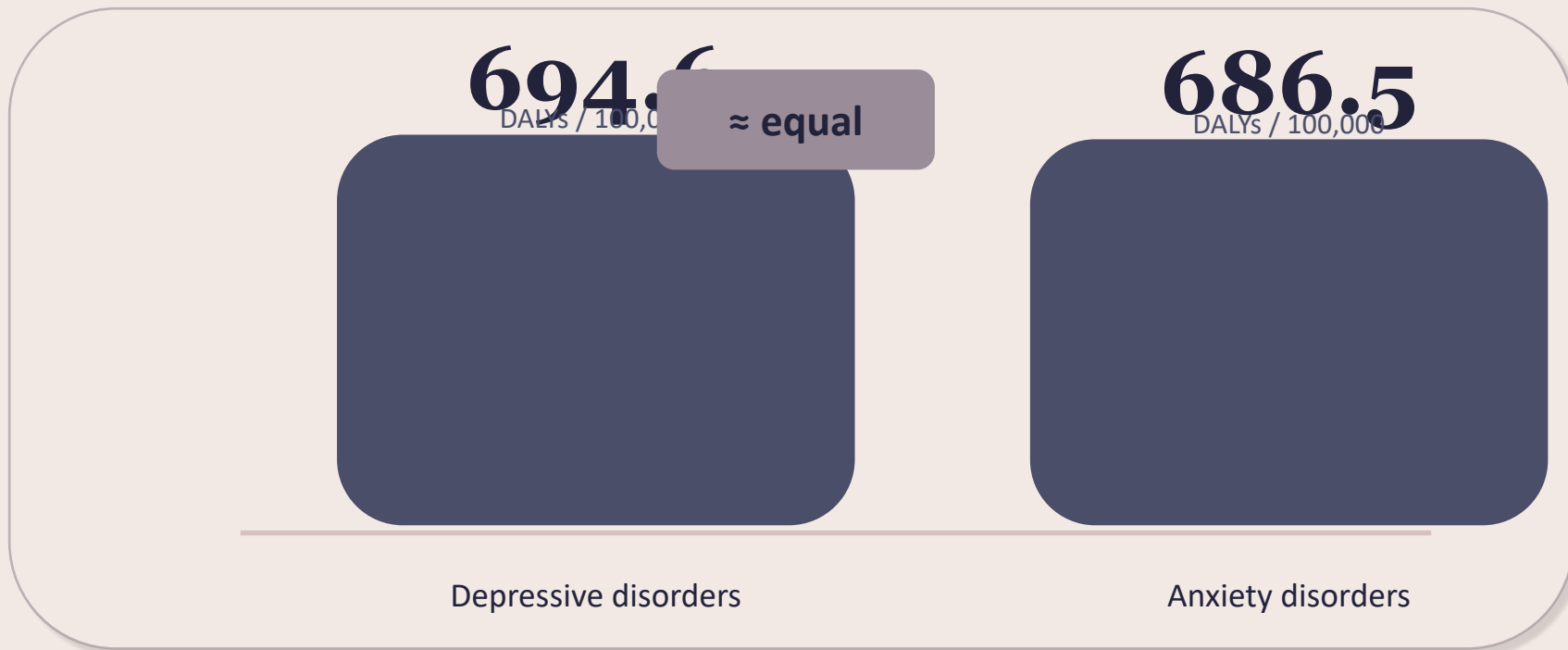
70
0

Depressive and anxiety disorders have nearly identical DALY rates and dominate the overall mental health

burden.
Source: Our World in Data (2023). Data: IHME GBD (2025).

Depression vs. anxiety: near-parity

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Together, depressive and anxiety disorders dominate the global mental health burden and dwarf other categories in DALYs.

Source: Our World in Data (2023). DALYs per 100,000 (World, 2023).

Severe conditions: smaller totals, high disability

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DALYs per 100,000 (World, 2023)

Depression

694.6

Anxiety

686.5

Schizophrenia

210.2

Bipolar

94.6

Schizophrenia and bipolar disorder contribute fewer DALYs at the population level, but are associated with severe disability for affected individuals.

Source: World Health Organization (2023). [DALYs by Cause, World, 2023](#).

Effective treatments exist, but many people do not receive adequate care.



Stigma

Discomfort disclosing symptoms to clinicians or even to people they know.



Access

Treatment is often lacking, especially in low- and middle-income countries.



Quality

Even when available, care may be of poor quality.



Underreporting

Disclosure barriers make true prevalence hard to estimate.

Implication for planning: scale access and quality while reducing stigma — and treat prevalence estimates as potentially understated.

Source: [Gentile et al. in PLoS \(2013\)](#) on treatment gap and data challenges.

Coping strategies vary widely

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Global surveys on dealing with anxiety or depression highlight diverse responses:



Prescribed medication

Common in higher-income settings



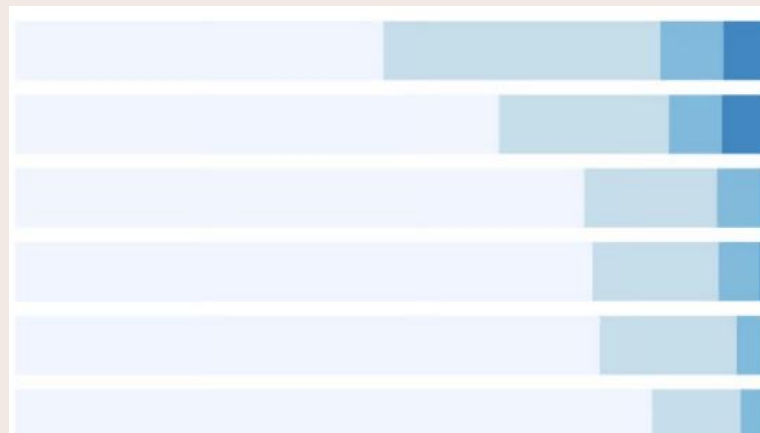
Religious / spiritual activities

Prevalent in many regions



Talking to friends or family

Widely reported



Policy implication: mental health responses are shaped by culture, economics, and health systems — avoid one-size-fits-all designs.

Source: Our World in Data (2023). Coping strategies survey (Figure 2).

What this means for public health planning

1

Scale common-disorder care

Depression and anxiety dominate mental health DALYs worldwide; prioritize evidence-based, scalable interventions.

2

Protect severe-condition services

Schizophrenia and bipolar contribute fewer DALYs overall, but carry high disability for individuals; maintain specialized care.

3

Close the treatment gap

Treatments exist, yet access and quality are often inadequate—especially where resources are limited.

4

Reduce stigma; improve measurement

Discomfort disclosing symptoms suppresses help-seeking and makes prevalence hard to estimate; expect underreporting.

5

Design for context

Coping strategies vary widely (medication, faith, social support); tailor programs to culture and system capacity.